DE QUERVAIN'S TENOSYNOVITIS

Background

- De Quervain's tenosynovitis is an overuse injury to the abductor pollicis longus (APL) and extensor pollicis brevis (EPB) tendons in the thumb.
- De Quervain's tenosynovitis is the most common source of wrist pain in adults. It's more common in women aged 30–50, and particularly in women who are recently postpartum. This is partly due to frequently picking up their baby. Other overuse activities can include gardening, playing racquet sports, and golfing.
- Symptoms include wrist pain at the base of the thumb that worsens with thumb and wrist movement, gripping, or making a fist. It usually affects the dominant hand due to activities that involve moving the thumb outward.
- Diagnosis is made through review of your history, physical exam, and sometimes imaging such as x-rays or an ultrasound.
- Treatment for De Quervain's tenosynovitis typically includes rest, splinting, ice therapy, NSAIDs, and physical therapy to reduce pain and inflammation. For more severe cases, corticosteroid injections or surgery may be necessary, followed by rehab.

Latest Research

- Immobilizing the thumb for 1–4 weeks with a thumb splint is a key part of treatment.
- Rehab can help strengthen the wrist, especially after a period of immobilization.
- Corticosteroid injections are safe and effective for long-term pain relief when symptoms continue despite immobilization and therapy.

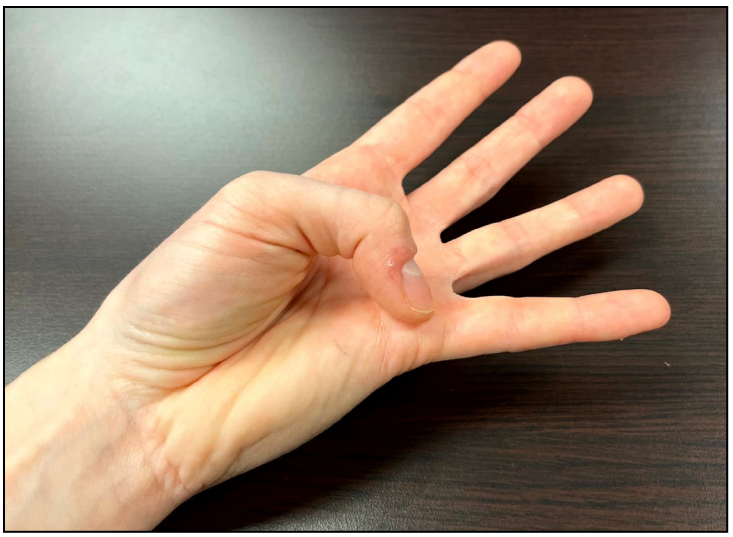


Rehab Instructions

Opposition Stretch:

1. Rest your injured hand on a table, palm up.
2. Touch the tip of your thumb to the tip of your little finger and gently slide your thumb toward your palm.
3. Hold for 10–15 seconds

Complete 3 sets of 10 reps 1 x a day.



Eccentric Finger Spring:

1. Put a rubber band around your fingers and injured thumb.
2. Open your hand to stretch the band, then slowly bring your thumb back to the starting position.
3. If this is painful, use a thinner or lighter rubber band.

Complete 3 sets of 10 reps 1 x a day.





Eccentric Wrist Radial Deviation:

1. Hold a water bottle or light dumbbell (~1–2 lbs) in the affected hand and place your forearm on a flat surface. **(C1)**
2. Use your other hand to lift up your affected wrist.
3. Let go and lower your affected hand back to the starting position in a slow and controlled motion.
4. Keep your forearm still during this exercise.
5. If this is painful, use less weight (for example, empty some water from the water bottle). Gradually increase the weight as the symptoms improve.

Complete 3 sets of 15 reps 1 x a day.





Practical Tips For Success

- Do these exercises in order, and go on to the next one when you can complete the exercise you're on pain free.
- Eccentric (tendon lengthening) exercises are essential for long-term improvement.
- Avoid repetitive activities (for example, carrying heavy grocery bags, playing racquet sports) that can overuse your thumb and wrist until your symptoms improve.
- If you have to do these activities, wear your splint while doing them.
- Recovery with splinting and therapy generally takes 6–12 weeks, though it can take longer in some cases.
- Your symptoms may recur if you can't change your activity level.

References

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